



ORBISMED

Confirmation of Clinical Necessity

Dwaine Chesmain

(Passport No. 564380765)

The treatment plan was devised based on his clinical needs following a detailed assessment, which revealed significant oral health issues requiring intervention. Below is a summary of the procedures performed and their clinical justifications:

❖ Closed Sinus Surgery

Sinus augmentation was performed to increase bone height in the posterior maxilla at sites **14, 16, 24, and 26**. This procedure was essential to provide sufficient bone volume for implant anchorage, ensuring biomechanical stability and osseointegration in areas of significant vertical bone loss.

❖ Implant Placement

Implants were strategically placed in the following positions based on the following oral health needs and bone density evaluations:

IMPLANT NECESSITY TABLE	
MAXILLA	MANDIBLE
12, 14, 16, 22, 24, 26	32, 34, 36, 42, 44, 46
To restore function and aesthetics in the maxillary arch, where tooth loss had led to compromised occlusion and chewing efficiency.	To rehabilitate the mandibular arch, providing stability and distributing occlusal forces evenly to prevent further bone loss.

❖ All-on-X Temporary Prosthesis

Following implant placement, an immediate load, screw-retained **All-on-X temporary prosthesis** was delivered. This full-arch prosthetic solution utilized a strategic distribution of implants to maximize mechanical leverage and ensure primary stability. The immediate provisional prosthesis serves several critical functions:

- Maintenance of occlusal vertical dimension and esthetics during the osseointegration period.
- Promotion of soft tissue healing and peri-implant mucosal adaptation.
- Distribution of functional loads to prevent micromovement of the implants, thereby supporting optimal bone-implant interface stability.

➤ This comprehensive treatment was deemed clinically necessary to address:

- Extensive edentulism, resulting in compromised masticatory efficiency and facial support.
- Prevention of progressive alveolar ridge resorption and maintenance of jawbone volume.
- Restoration of full-arch functionality and improved patient quality of life through enhanced oral biomechanics.

OrbisMed Clinics Coordinator :

Gulain Ozer

Date:

11.11.2024

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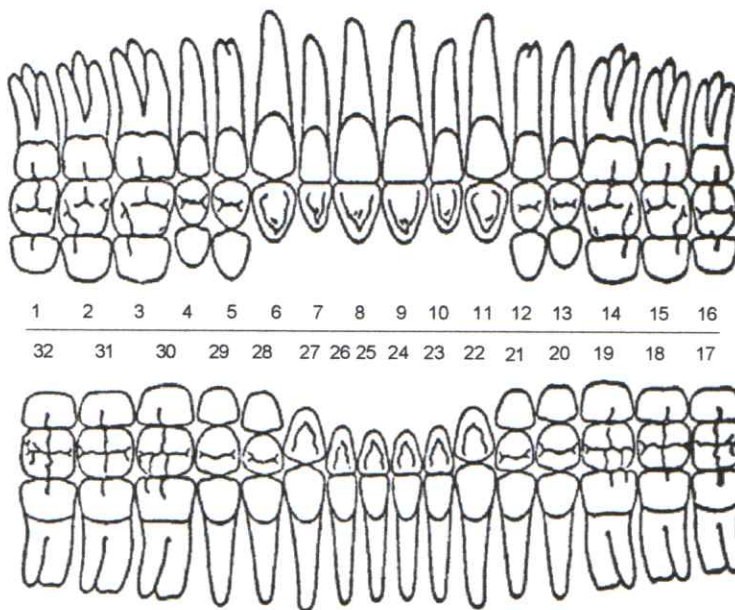
Implant Placement

DENTAL TREATMENT PLAN

ORBISMED CLINICS

SECTION I - PLANNED TREATMENT AND SEQUENCE OF ACCOMPLISHMENT

CHART



Maxilla: 12, 14, 16, 22, 24, 26

Mandible: 32, 34, 36, 42, 44, 46

12, 14, 16, 22, 24, 26: Restoration of the posterior and anterior maxilla to reestablish occlusal function and provide a robust prosthetic platform.

32, 34, 36, 42, 44, 46: Rehabilitation of the posterior and anterior mandible to restore full-arch functionality, optimize occlusal load distribution, and prevent further alveolar bone atrophy.

REMARKS (If appropriate):

DATE:

11.11.2024

CLINIC COORDINATOR NAME:

Gülain Özer

CLINIC COORDINATOR SIGNATURE

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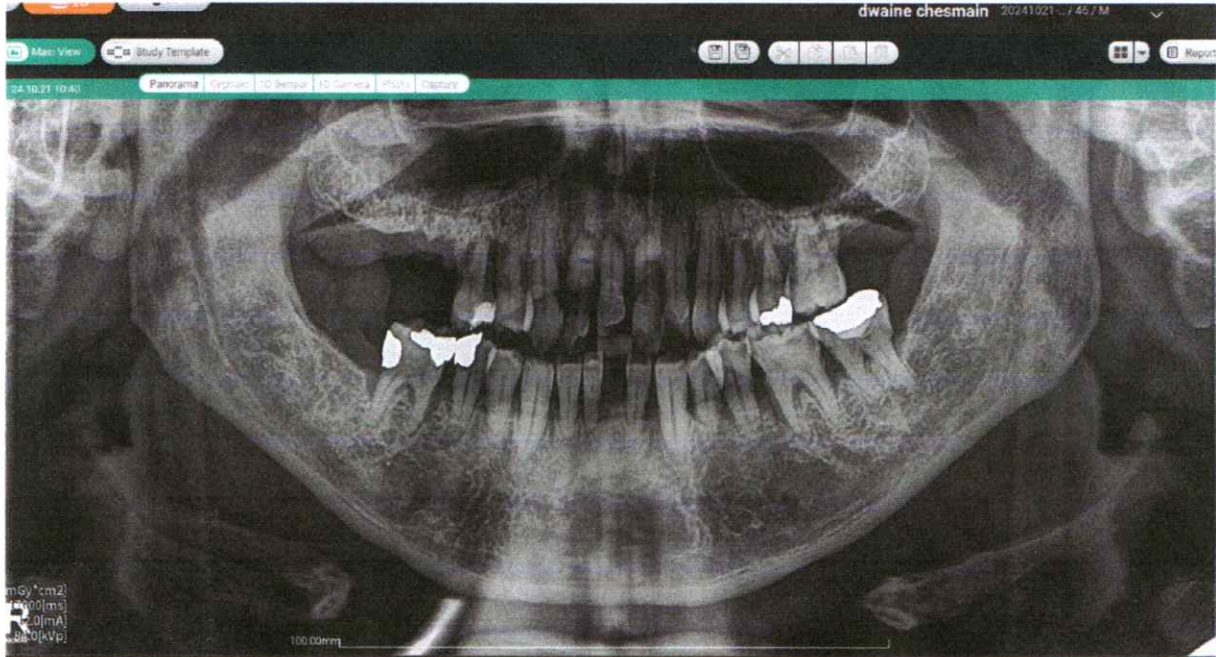
Before-And-After X-Ray

Dwaine Chesmain

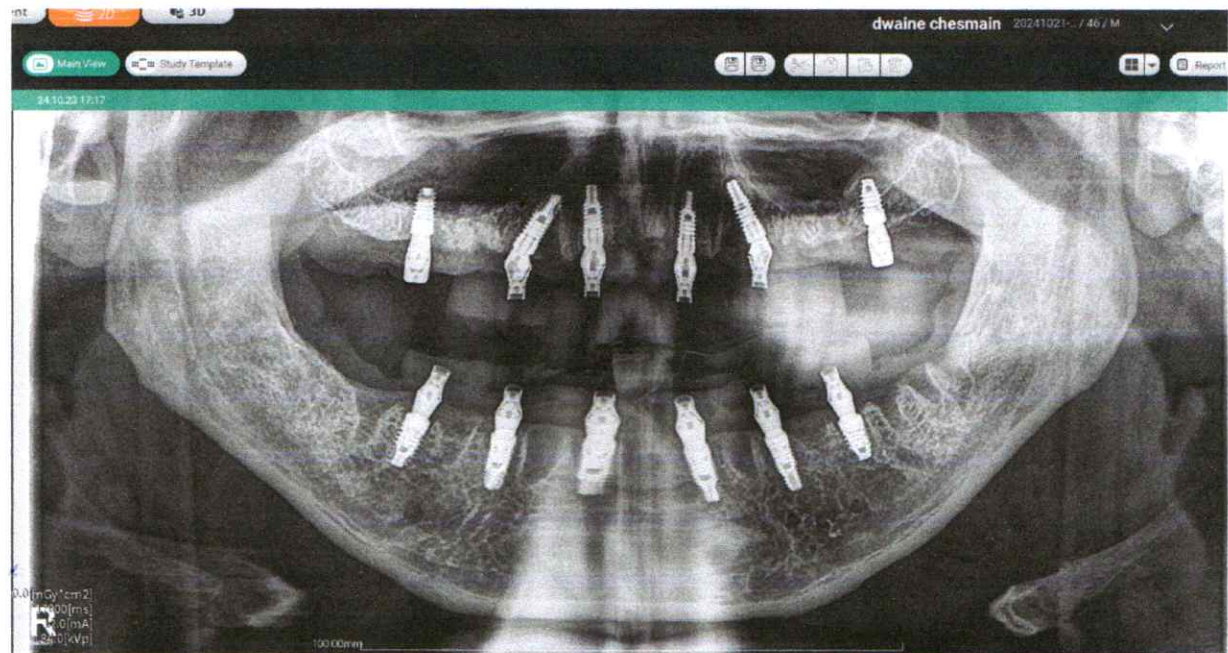
(Passport No. 564380765)

Included below are the pre-operative CBCT scan and post-operative panoramic radiograph. These images demonstrate the initial oral condition and verify the precise placement and alignment of the dental implants following treatment.

24.10.21 10:40 Before X-RAY



23.10.24 17:17 After X-Ray



OrbisMed Clinics Coordinator

Gillian Osier

Date:

11.11.2024

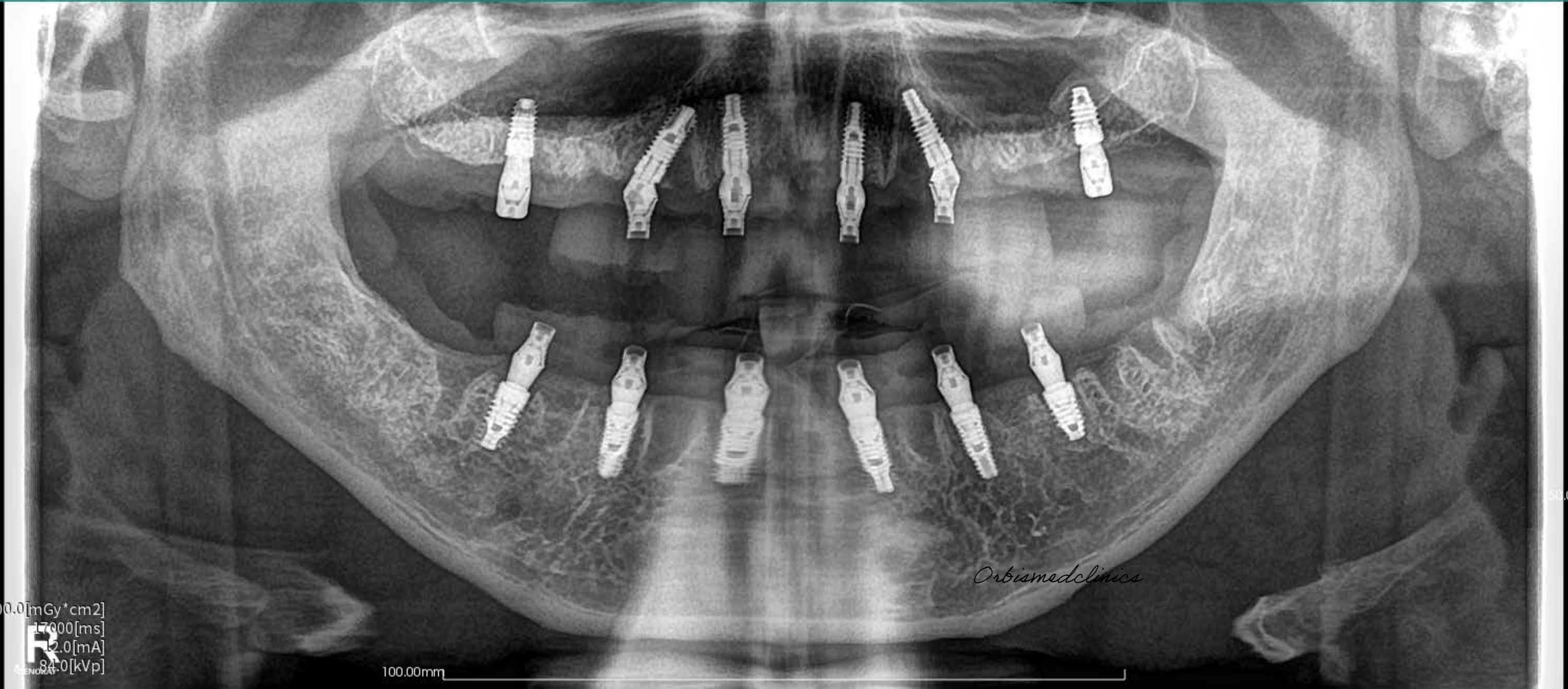
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